TCGA-18-3409

Surgical Pathology Consultation Report

SPECIMEN(S) RECEIVED
1. Lung: LEFT UPPER LOBE - QUICK SECTION
2. Lymph node: INF PULM ligament S
3. Lymph node: Right TB angle
4. Lymph node: Interlobar
5. Lymph node: Interlobar ST11R no.2
   Lymph node: Lobar ST12R/QS
   Lymph node: Right TB angle
8. LUNGS: Right upper lobe/QS
9. LUNGS: Wedge right upper lobe
10. Lymph node: Interlobar ST11L
11. Bone: 5th rib
12. LUNGS: Left upper lobe

DIAGNOSIS
1. Left upper lobe of lung wedge resection:
- Basaloid squamous cell carcinoma, 1.4 cm in greatest diameter.
- Stapled resection margins negative for tumor.

2. Inferior pulmonary ligament ST9R lymph node:
- Negative for malignancy, lymph node.

3. - 7. Right TB angle ST10R, interlobar ST11R, interlobar ST11R no.2, lobar ST12R, right TB angle ST10R:
- Negative for malignancy, lymph node. (x5)

8. Right upper lobe lobectomy:
- Basaloid squamous cell carcinoma, 1.1 cm in greatest diameter.
- Bronchial resection margin negative for tumor.
- One peribronchial lymph node negative for malignancy.

9. Right upper lobe wedge resection:
   Negative for malignancy.
- Normal lung.

Status: complete

[redacted]

[redacted]

10. Interlobar ST11L:
- Negative for malignancy, lymph node.

11. Bone 5th rib:
- Consistent with normal rib bone. (GROSS DIAGNOSIS ONLY)

12. Left upper lobe lobectomy:
- No residual malignancy.
- Bronchial resection margin negative for tumor.
- Four peribronchial lymph nodes negative for malignancy.

MMENT
It is interesting that the two tumors demonstrate similar histological appearances, yet they are almost certain to represent separate T1N0 primaries.

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CLINICAL HISTORY
LUNG CA

GROSS DESCRIPTION
1. The specimen container labeled with the patient' s name and as "left upper lobe q.s.", contains two pieces of lung tissue consisting of a wedge of lung and second smaller piece of tissue received fresh. The wedge of lung measuring 6.0 x 4.5 x 2.0 cm and the second piece of lung tissue measures 4.5 x 2.0 x 0.3 cm and this has staples. The second piece of tissue is taken from over to tumor (to allow access of surgeon for culture). The wedge of lung tissue has two stapled resection margins measuring 3.5 cm and 4.5 cm and an area of exposed lung parenchyma measuring 4.0 x 4.0 cm. This area is marked with India ink. On gross examination there is a greyish-white tumor measuring 1.4 x 1.3 x 0.9 cm. The tumor is well-circumscribed and is located immediately below the exposed lung parenchyma. A representative section of the tumor is examined at intraoperative consultation by frozen section. The rest of the lung parenchyma appears to be grossly unremarkable. Please that the rked area in India ink is not the true margin. Representative sections are submitted as follows:

1A frozen section resubmitted
1B shave resection margin (shorter stapled resection margin)
1C shave resection margin (longer stage resection margin)
1D-1F representative section of the tumor in relation to the exposed lung parenchyma with ink margin,
pleura and adjacent lung parenchyma
1G representative section of the lung parenchyma
1H -1I smaller piece of lung tissue serially sectioned and submitted in toto.

2. The specimen container labeled with the patient' s name and as nferior pulmonary ligament ST9 q.s.", contains three pieces of carbon laden tissue received fresh. The tissue measures from 0.6 to 1.3 cm in greatest dimension. This I=is examined at intraoperative consultation by frozen section on the entire specimen.
2A -2B frozen section resubmitted (block 2A the largest piece of tissue bisected).

3. The specimen container labeled with the patient' s name and as "right TB angle ST10R q.s.", contains a piece of carbon laden lymph node measuring 1.0 x 0.9 x 0.3 cm received fresh. This is examined at intraoperative consultation by frozen section on the entire specimen.
3A frozen section resubmitted (tissue bisected)

4. The specimen container labeled with the patient' s name and as "interlobar ST11R q.s.", contains four fragments of anthracotic tissue ranging in size from 0.4 to 0.8 cm in greatest dimension received fresh. This is examined at intraoperative consultation by frozen section on the entire specimen.
4A frozen section resubmitted.

5. The specimen container labeled with the patient' s name and as "interlobar ST11R number 2 q.s.", contains a piece of anthracotic tissue measuring 2.0 x 1.3 x 0.7 cm received fresh. The specimen is bisected and is examined at intraoperative consultation by frozen section on the entire specimen.
5A frozen section resubmitted.

6. The specimen container labeled with the patient' s name and as "lobar 12R q.s", contains a piece of anthracotic tissue measuring 2.0 x 1.0 x 0.2 cm received fresh. This is examined at intraoperative consultation

[redacted]

by frozen section on
the entire specimen.
6A frozen section resubmitted .

7. The specimen container labeled with the patient' s name and as "right TB angle ST10R q.s.", contains a piece of tissue measuring 1.7 x 0.6 x 0.3 cm received fresh. This is examined at intraoperative consultation by frozen section on the entire specimen.
7A frozen section resubmitted .

8. The specimen container labeled with the patient' s name and as "right per lobe q.s.", contains a lobectomy specimen received in 10% buffered formalin. The specimen measures 20.0 x 13.5 x 5.5 cm in its formalin inflated state. On gross examination the pleura appears smooth, shiny and intact. On sectioning there is a well-circumscribed blackish-tan tumor measuring 1.1 x 1.0 x 0.9 cm. The tumor is approximately 2.0 cm away from the bronchial resection margin. A representative section from the tumor is examined at intraoperative consultation by frozen section. No other nodules are identified grossly. The rest of the lung parenchyma appears to be slightly emphysematous. One lobar lymph node is identified grossly measuring 0.6 x 0.6 cm. The rest of the lung parenchyma appears to be slightly emphysematous. Representative sections are submitted as follows:
8A frozen section resubmitted
8B bronchial resection margin
8C vascular resection margin
8D-8F the rest of the tumor serially sectioned and submitted in toto
8G-8H representative section of the lung parenchyma
8I one lobar lymph node

9. The specimen container labeled with the patient' s name and as "wedge right upper lobe", contains a wedge of lung tissue received in 10% buffered formalin. The tissue measures 6.2 x 3.0 x 2.0 cm. Two stapled resection margin is identified measuring 3.9 and 6.0 cm long. On gross examination the pleura is smooth, shiny and intact. On sectioning no nodule is identified grossly. The lung parenchyma is slightly emphysematous. The specimen is submitted in toto after stapled margins are removed as follows:
9A shave resection margin (shorter staple resection margin)
shave resection margin (longer staple resection margin)
9C-9G the rest of the lung parenchyma serially sectioned and submitted

[redacted]

[redacted]

in toto.

10. The specimen container labeled with the patient' s name and as "interlobar ST11L", contains a single piece of blackish-tan tissue received in 10% buffered formalin. The tissue measures 0.8 x 0.8 x 0.3 cm.
10 specimen submitted in toto.

11 The specimen container labeled with the patient' s name and as "5th rib", contains a piece of bone with soft attached received in 10% buffered formalin. The bone measures 2.5 x 1.0 x 0.4 cm. No gross normality is noted. No tissue is submitted for histologic examination.

Gross Diagnosis: Bone 5th rib.

12 The specimen container labeled with the patient' s name and as "left upper lobe", contains a lobe of lung received in 10% buffered formalin. The tissue measures 9.0 x 6.3 x 3.0 cm. One stapled resection margin is identified measuring 8.5 cm long and one sutured resection margin is identified measuring 4.0 cm long. On gross examination the pleura appears to be smooth, tiny and intact. On sectioning, the lung parenchyma is slightly emphysematous. No nodule is identified grossly. Multiple lobar lymph nodes are identified grossly ranging in size from 0.3 x 0.3 to 0.4 x 0.4 cm. Two pieces of lung tissue are taken for snap frozen. Representative sections are submitted as follows:
12A bronchial resection margin
12B shave resection margin from stapled margin
12C shave resection margin from the stitch resection margin
12D -2G representative section of the lung parenchyma
12H four lobar lymph nodes.

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QUICK SECTION
1A "Non-small cell carcinoma, favor squamous cell carcinoma"
2A-2B "Negative for malignancy".
3A "Negative for malignancy".
4A "Negative for malignancy".
5A "Negative for malignancy".
[illegible]A,7A "Negative for malignancy".
[illegible] "Non small cell carcinoma favor squamous cell carcinoma".

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[redacted]

[redacted]